



Sample #

Gender: Female

DOB:

Race: Black

Report Date:

Pathology Report:

Criteria	Yes	No
Diagnosis Discrepancy		☒
Primary Tumor Site Discrepancy		☒
H/AA Discrepancy		☒
Prior Malignancy History		☒
Dual/Synchronous Primary Notes		☒
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer: <i>PMI</i>	Date Reviewed: <i>12/29/11</i>	

Surgical Pathology Report

FINAL PATHOLOGIC DIAGNOSIS

A. Right pelvic lymph nodes; dissection:

- Fourteen lymph nodes, no tumor (0/14).

B. Left pelvic lymph nodes; dissection:

- Twenty nine lymph nodes, no tumor (0/29).

C. Right ureter; excision:

- Portion of ureter, no tumor.

D. Bilateral ovaries, left fallopian tube and uterus; hysterectomy and salpingo-oophorectomy:

- Uterus with benign secretory endometrium, adenomyosis, and multiple leiomyomas (largest 6 cm).
- Bilateral ovaries with physiologic changes.
- Left fallopian tube, no tumor.

E. Urinary bladder and round ligament; cystectomy:

- Invasive high grade urothelial carcinoma with extensive squamous differentiation, see pathologic parameters.

F. Right pelvic lymph nodes; dissection:

- Fifteen lymph nodes, no tumor (0/15).

Urothelial Carcinoma Pathologic Parameters

1. Tumor type: Invasive urothelial carcinoma with extensive squamous differentiation.
2. Grade of tumor: High grade.
3. Depth of invasion: Muscularis propria, outer half.
4. Tumor distribution: Solitary, anterior wall, 6.0 cm exophytic mass.
5. Ureteral margins: Negative for tumor.
6. Distal urethral margin: Negative for tumor.
7. Soft tissue margin or serosa: Negative for tumor.
8. Lymph nodes: Negative for tumor (0/58).
9. pTNM: pT2b,N0,MX.

Effective . this Checklist utilizes the 7th edition TNM staging system for Bladder of the American Joint Committee on Cancer (AJCC) and the International Union Against Cancer (UICC).

10D-0-3

carcinoma, urothelial, NOS 8120/3

Path Site: bladder, NOS C67.9

CQCF bladder, wall, anterior C67.3

1/12/12

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[], MD

Interpretation performed by the Attending Pathologist and reviewed with the Resident/Fellow, [], M.D., Ph.D.

Electronically Signed Out by [], MD

Clinical History:

The patient is a -year-old female with history of bladder cancer undergoing cystectomy.

Specimens Received:

A: Right pelvic lymph nodes

B: Left pelvic lymph nodes

C: Right distal ureter

D: Ovaries, tubes and uterus

E: Bladder and round ligament

F: Right pelvic lymph node

Gross Description:

The specimens are received in six containers each labeled with the patient's name and medical record number.

A. The first container is additionally identified as, "right pelvic lymph nodes". Received fresh for intraoperative diagnosis is a 6.5 x 4.8 x 1.5 cm aggregate of lobulated yellow fibrofatty tissue. The specimen is dissected for lymph nodes to reveal multiple apparent lymph nodes ranging from 0.2-2.5 cm in greatest dimension. One representative lymph node is submitted for frozen section and read as "reactive lymph node, NEOT" by Dr. []. The frozen section remnant is submitted as A1 FS. The remaining apparent lymph nodes are submitted as follows: A2-4: 2 lymph nodes each cassette; A5-6: 4 lymph nodes each cassette

B. The second container is additionally identified as, "left pelvic lymph node". Received fresh for intraoperative diagnosis is a 6.5 x 4.5 x 1.0 cm aggregate of lobulated yellow fibroadipose tissue. The specimen is dissected for lymph nodes to reveal multiple appearing lymph nodes ranging from 0.8-3.0 cm in greatest dimension. Representative sections of a small lymph node and a large node are submitted for frozen section and read as "2 active lymph nodes, NEOT" by Dr. [] The frozen section remnant is submitted as B1 FS and B2 FS. The remaining appearing lymph nodes are submitted as follows: B3: Remaining portion of large lymph node; B4: Remaining portion of the small lymph node; B5-7: 5 lymph nodes each cassette; B8: 7 lymph nodes; B9: One lymph node bisected; B10: One lymph node sectioned; B11-12: One lymph node sectioned

C. The third container is additionally identified as, "right distal ureter". Received fresh and placed in formalin is a 0.5 cm unoriented segment of ureter. The specimen is sectioned and entirely submitted as C1.

D. The fourth container is additionally identified as, "uterus, tubes, ovaries". Received fresh and placed in formalin is a hysterectomy specimen, weighs 479.1 gm with bilateral adnexa. The uterus measures 7.8 cm from fundus to

lower uterine segment by 7.5 cm from cornu to cornu by 11.3 cm from anterior to posterior. The serosal surface is red-tan, and glistening with a prominent serosal bulge (2.4 cm in greatest dimension, at anterior/fundal aspect). The cervix is not present with the specimen. The anterior surface is inked blue and the posterior surface is inked black. The specimen is opened revealing a 0.9 cm endocervical canal remnant lined with tan-pink rugated mucosa and an endometrial cavity distorted by submucosal nodules measuring 0.5 cm from cornu to cornu by 6.5 cm in length and lined with hemorrhagic and thickened, endometrium measuring 0.2 cm in thickness. The specimen is sectioned revealing pink-tan myometrium which is distorted on the anterior, posterior and fundus with multiple, rubbery and whorled nodules measuring from 0.7-6.0 cm. On cut section the nodules are devoid of hemorrhage, calcifications, necrosis, or hemorrhage. The myometrium has a maximum wall thickness of 3.5 cm and is coarsely trabeculated. The right adnexa consist of a 4.5 cm long and 3.0 cm wide portion of soft tissue. The fallopian tube with fimbriated end is not identified. The serosa is pink-red and glistening. The attached white-tan, cribriform ovary measures 4.5 x 2.5 x 1.6 cm and is sectioned to revealed white dense ovarian stroma with a 2.5 cm hemorrhagic cyst filled with semisolid material. The left adnexa consist of a 3.5 cm long and 2.5 cm diameter fimbriated fallopian tube. The serosa is pink-red and glistening. The specimen is serially sectioned revealing a pinpoint lumen and a 0.4 cm wall thickness. The attached white-tan, cribriform ovary measures 3.8 x 2.1 x 1.3 cm and is sectioned to revealed fibrotic white and ovarian stroma with few small cysts measuring up to 2.5 cm in greatest dimension and filled with hemorrhagic solid material.. Representative sections are submitted as follows:

- D1: Anterior cervix remnant, entirely submitted
- D2: Posterior cervix remnant, entirely submitted
- D3-4: Anterior endomyometrium (D3: With nodules, D4: Full thickness)
- D5-6: Posterior endomyometrium
- D7: Whorled nodules
- D8: Right ovary and right adnexal soft tissue
- D9: Left fallopian tube
- D10: Left ovary and surrounding soft tissue

E. The fifth container is additionally identified as, "bladder and round ligament". Received fresh and placed in formalin is a 61.9 g, 17 x 13.5 x 3.7 cm cystectomy specimen with attached mesenteric fat. The bladder measures 9 x 7 x 3 cm. The right ureteral stump measures 2.2 x 0.6 cm and the left 1.0 x 0.7 cm, and both demonstrate patent lumina. The urethral margin is inked black. The right half of the bladder is inked blue and the left half is inked black. Also received in the same container are 2 detached tubular structures, measuring 2.8 x 1.0 cm and 4.3 x 0.6 cm.

The bladder is opened laterally along the urethra. The bladder demonstrates a 6.0 x 5.8 x 2.0 cm exophytic, pink-tan, friable mass attached to the anterior wall and 2.3 cm from the urethral margin. Serial sectioning reveals tumor invasion into the submucosa to a depth of 0.5 cm, extending to 1.5 cm of the inked margin. There is a 2.0 x 1.5 cm ill-defined area of plaque-like raised mucosa identified in the posterior wall, about 2 cm lateral to the aforementioned tumor.

The remainder of the mucosa is tan-brown, edematous, and glistening with a uniform 1.2 cm wall thickness. The bilateral ureteral orifices are identified and probe patent. Gross photographs are taken. Representative sections are

submitted as follows:

- E1: Left ureteral margin
- E2: Right ureteral margin
- E3: Distal urethral margin
- E4-5: Mass at deepest invasion (full thickness, tandem sections)
- E6-10: Multiple tumor sections, and surrounding normal looking mucosa
- E11: Raised lesion on the posterior wall
- E12: Uninvolved dome
- E13: Uninvolved posterior wall
- E14: Uninvolved right wall
- E15: Uninvolved left wall
- E16: Uninvolved trigone

F. The sixth container is additionally identified as, "right pelvic lymph node". Received fresh and placed in formalin is a 7.0 x 2.5 x 2.5 cm aggregate of lobulated yellow fibrofatty tissue. The specimen is dissected for lymph nodes to reveal multiple apparent lymph nodes ranging from 1.0-5.0 cm in greatest dimension. All apparent lymph nodes are submitted as follows: F1-3: 5 lymph nodes each cassette, F4: One lymph node bisected; F5-6: One lymph node sectioned

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[], M.D., Ph.D.

Intraoperative Consult Diagnosis:

A1 FS: Reactive lymph node, NEOT

B1 and B2 FS: 2 reactive lymph nodes, NEOT